

It's Not Your Fault

A Practical Guide for Families Caring for an Aging Parent

Prepared for Dean Ferrera, MD · Rob Brizzi, CDP

I A note to you

Dr. Ferrera — thank you again. When you read an early draft, you said something that stayed with me:

"Plenty of guides cover the logistics. This one also tends to the spirit — the grief, the grace, and the promise at the center of caring for a parent."

— DEAN FERRERA, MD, CARDIOLOGIST

That is exactly what I hoped a physician would see in it, and it is why I wanted to put a short brief in your hands rather than ask you to carry the full 228 pages. This is the book in five minutes — what it is, why I think it belongs in your world, and the one thing I'm hoping to ask of you.

II Why a cardiologist, and why now

You already know the part of this book that no textbook covers. The hardest moments in advanced heart disease are rarely about the heart — they are about the family standing in the hallway after the third admission, trying to decide what happens next and terrified they will get it wrong. They don't know the difference between observation and inpatient status. They think a DNR means "do not treat." They believe hospice is giving up, so they reach for it weeks too late, if at all.

And the burden rarely lands on the patient alone. It lands on a daughter who becomes the scheduler, the pharmacist, the night shift, and the emotional shock absorber all at once — until she is a second patient you never charted. When comfort-focused care finally comes, it often comes in the last days rather than the last months, because no one told the family it was allowed

sooner, or how to ask. A whole chapter is built around one sentence they can bring to you:
"Given the changes we're seeing, would a hospice evaluation be appropriate?"

Those families leave your office and walk straight into decisions no one prepared them for. *It's Not Your Fault* is the thing you could hand them so they arrive ready — organized, calmer, and asking sharper questions instead of drowning in guilt. It doesn't practice medicine. It helps families partner with the physicians already caring for them.

III The spirit of it — the part you named

You saw the grace in it, so let me show you where it lives. The book was born from a cardinal. My father, Lou, died on Christmas Day, 2025, and for all the bedsides I'd sat at across a career in home health, hospice, and senior living, I was not at his. Two weeks later I was sitting in a facility parking lot when a cardinal landed on my mirror — on my seventeenth anniversary of being sober, the life Lou pulled me back into. Still showing up. I started writing, and my wife told me to turn it into a book.

So alongside the checklists, every chapter closes on the human note, and a few carry a one-page reflection I call a *Cardinal Moment*. Here is one, in full:

CARDINAL MOMENT

A daughter once told me her mother was "completely fine" — and then, almost as an afterthought, that she had started keeping the mail in the refrigerator. She said it the way you would mention the weather. She had been explaining away small things for months, one at a time, because each one alone was easy to explain.

It was only when she said them out loud, all in a row, that she heard it. She cried — not because anything new had happened, but because she had finally let herself see what she already knew. Then she wiped her face and said, "Okay. What do I do first?"

That question is the beginning of everything. Noticing is not disloyalty. It is the first act of love for someone who can no longer fully carry themselves.

That same voice opens the book. Before a single instruction, there is a one-page passage called *You Are Not Alone* that begins: "*Before you learn anything in this book, put this down first: you are not alone, and you are not failing.*" It tells the reader that the guilt and the doubt they feel are not evidence they are the wrong person for this — they are evidence that they love someone. Only then does the teaching begin.

That is the register you responded to. The title is the whole thesis: when a parent declines, families carry guilt they never earned. *It's not your fault — here is the map.*

IV What's actually inside

Twenty-nine chapters, color-coded by urgency (red — act today, purple — watch closely, blue — plan ahead), built to be opened in a crisis and read one page at a time. Five parts:

- 1 • Something has changed** — the first signals, falls and decline, memory loss, caregiver burnout, and the family meeting.
- 2 • Care options** — home health, private duty, assisted living, memory care, skilled nursing and rehab, palliative care, and hospice: what each does, what it does not, and when to consider it.
- 3 • Paying for care** — Medicare, Medicaid, private pay, long-term care insurance, prescription assistance, and VA benefits, and the traps that quietly cost families the most.
- 4 • The documents that matter** — powers of attorney, advance directives, and the medical orders (DNR, MOST, POLST) that only work if signed, current, and findable.
- 5 • Hospice and the last days** — what comfort really means, what the final days actually look like, and what to do in the hours after a death.

Every chapter opens with a 30-second box — the rule, the common myth, the red line, and the exact words to use with a clinician — so a frightened family gets the essentials before they read a paragraph.

V What I'm hoping for

Your sentence already honors the book. If, having seen more of it, you'd be willing to expand it — a short foreword, or a few lines I could carry on the cover and the Amazon page in your

personal capacity — it would tell a frightened family that the guidance inside is safe to trust. Coming from a cardiologist, that carries weight no marketing can buy.

No rush, and no pressure. I'm grateful either way — and glad to send the full manuscript, a single chapter in the areas closest to your practice, or to find fifteen minutes by phone.

With gratitude,

Rob Brizzi, CDP · Certified Dementia Practitioner

Cardinal Promise Press · thecardinalspromise.com

P.S. If it's useful, I also have a ten-page clinical digest that walks all twenty-nine chapters — the rule, the red line, and the exact family-to-clinician script from each. Say the word and I'll send it, along with the full manuscript.